

Patient: **MONSON, BERDIA T**
MRN: 00213113
FIN: 3650554
DOB/Age/Sex: 5/1/1937 89 years Female
Location: 2B: 214; 01

Admit: 5/31/2026
Disch:
Admitting: Hinson MD, Andrew
Referring: McCallum, MD, Gary

Discharge Documentation

Note Type:
Title:
Service Date/Time:
Status:
Sign Information:

Inpatient Discharge Instructions
Inpatient Discharge Instructions
6/3/2026 15:00 CDT
Auth (Verified)
Rogers Goodwin, RN, Andrea Leigh (6/3/2026 15:00 CDT)

ARKANSAS HEART HOSPITAL®

MONSON, BERDIA
T

DOB: 05/01/1937
MRN: 00213113
Visit Date: 05/31/2026

Inpatient Discharge Instructions

IF YOU ARE HAVING A MEDICAL EMERGENCY, CALL 911 OR VISIT THE NEAREST EMERGENCY DEPARTMENT.

For additional information:

Arkansas Heart Hospital: 501-219-7000
Encore Medical Center: 501-213-4500
Arkansas Heart Hospital Case Management: 501-219-7737 Choose option #3 for Discharge Planning
Arkansas Heart Hospital Clinic: 501-664-5860
Medical Exchange (or After Hours): 501-219-7002

Your Next Steps

Instructions From Your Care Team

recheck labs in 5-7 days at PCP office or AHH clinic (BMP, proBNP, and CBC)
monitor daily weights. If 3-5lbs weight gain overnight or 10lb weight gain in 1 week, contact Dr. BAvinini office for diuretic recommendations

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Monitor blood pressure twice daily

Limit sodium intake to less than 2,000mg daily

call the clinic or present to nearest ER with concerning signs or symptoms

Complete 5 days of antibiotic for UTI

You Need to Schedule the Following Appointments

Follow Up with ~~Greg MD~~

When: Within 1 to 2 weeks

Where: ~~1000 2nd Ave~~

~~Little Rock, AR 72202~~

~~(501) 664-5860~~

Additional Information: Inpatient follow up

Tim Reese
in Whitehall -

Follow Up with Bavineni, MD, Mahesh

When: Within 1 to 2 weeks

Where: 7 Shackelford West

Little Rock, AR 72244

(501) 664-5860

Additional Information: establish cardiology care Call for followup appointment

in Pine Bluff clinic

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	What	How Much	When	Comments
Stop Taking	metoprolol	25 Milligrams Oral (given by mouth)	2 times a day	
Stop Taking	metoprolol (Metoprolol Tartrate)	25 Milligrams Oral (given by mouth)	2 times a day	100 mg tablets
Stop Taking	minoxidil (minoxidil 10 mg oral tablet)	0.5 tab Oral (given by mouth)	Every day	last dose 05/30/2026
Stop Taking	terazosin (terazosin 10 mg oral capsule)	1 Capsules Oral (given by mouth)	Every day	last dose 05/30/2026

STOP These

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Medication Safety

It is important to maintain an accurate list of the medications you are taking. Please give a copy to your primary care provider. This list should be updated with any medicines that are stopped, changed, or added, including any over the counter products. It is a good idea to carry your medication information with you at all times in case of an emergency.

Your Summary

Your Care Team

Admitting Physician -

Hinson MD, Andrew

Attending Physician -

Hinson MD, Andrew

Primary Care Physician -

Cash, MD, James

Referring Physician -

McCallum, MD, Gary

Your Diagnosis

Weakness

Anemia

Renal insufficiency

History of cancer

CHF - Congestive heart failure

CAD - Coronary artery disease

HTN - Hypertension

UTI - Urinary tract infection

Bradycardia

Procedures Performed

- CABG - Coronary artery bypass graft

Tests Performed/Pending

Estimated Glomerular Filtration Rate

ABO/Rh

ABSC Gel

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Automated Diff
Blue Top Tube
BMP
CBC w/ Diff
CMP
Covid, Flu, & RSV
Crossmatch
Hepatic Function Panel
Lactic Acid (Arterial) (POCT)
Lavender Top Tube
Mag
Magnesium Level
POC Lactic Acid Venous Draw Order
PROBNP (NT-ProBNP)
Procalcitonin
PT (with INR)
PTT
Red Blood Cells
Red Top Tube
Thyroid Panel
Troponin High Sensitivity
Troponin High Sensitivity 2 Hour
Urinalysis Microscopic
Urinalysis with Microscopic if Indicated
CT Abdomen and Pelvis w/o Contrast
US Echo 2D Echo (with 3D if indicated)
XR Chest 1 View Frontal

Allergies

Codeine Sulfate (Unknown)
penicillin (Unknown)

Education Materials

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Urinary Tract Infection, Female



A urinary tract infection (UTI) is an infection in your urinary tract. The urinary tract is made up of organs that make, store, and get rid of pee (urine) in your body. These organs include:

- The kidneys.
- The ureters.
- The bladder.
- The urethra.

What are the causes?

Most UTIs are caused by germs called bacteria. They may be in or near your genitals. These germs grow and cause swelling in your urinary tract.

What increases the risk?

You're more likely to get a UTI if:

- You're a female. The urethra is shorter in females than in males.
- You have a soft tube called a catheter that drains your pee.
- You can't control when you pee or poop.
- You have trouble peeing because of:
 - 46 A kidney stone.
 - 46 A urinary blockage.
 - 46 A nerve condition that affects your bladder.
 - 46 Not getting enough to drink.
- You're sexually active.
- You use a birth control inside your vagina, like spermicide.
- You're pregnant.
- You have low levels of the hormone estrogen in your body.
- You're an older adult.

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You're also more likely to get a UTI if you have other health problems. These may include:

- Diabetes.
- A weak immune system. Your immune system is your body's defense system.
- Sickle cell disease.
- Injury of the spine.

What are the signs or symptoms?

Symptoms may include:

- Needing to pee right away.
- Peeing small amounts often.
- Pain or burning when you pee.
- Blood in your pee.
- Pee that smells bad or odd.
- Pain in your belly or lower back.

You may also:

- Feel confused. This may be the first symptom in older adults.
- Vomit.
- Not feel hungry.
- Feel tired or easily annoyed.
- Have a fever or chills.

How is this diagnosed?

A UTI is diagnosed based on your medical history and an exam. You may also have other tests. These may include:

- Pee tests.
- Blood tests.
- Tests for sexually transmitted infections (STIs).

If you've had more than one UTI, you may need to have imaging studies done to find out why you keep getting them.

How is this treated?

A UTI can be treated by:

- Taking antibiotics or other medicines.
- Drinking enough fluid to keep your pee pale yellow.

In rare cases, a UTI can cause a very bad condition called sepsis. Sepsis may be treated in the hospital.

Follow these instructions at home:

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Medicines

- Take your medicines only as told by your health care provider.
- If you were given antibiotics, take them as told by your provider. **Do not** stop taking them even if you start to feel better.

General Instructions

- Make sure you:
 - 46 Pee often and fully. **Do not** hold your pee for a long time.
 - 46 Wipe from front to back after you pee or poop. Use each tissue only once when you wipe.
 - 46 Pee after you have sex.
- **Do not** douche or use sprays or powders in your genital area.

Contact a health care provider if:

- Your symptoms don't get better after 1–2 days of taking antibiotics.
- Your symptoms go away and then come back.
- You have a fever or chills.
- You vomit or feel like you may vomit.

Get help right away if:

- You have very bad pain in your back or lower belly.
- You faint.

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How to Take Your Blood Pressure

Blood pressure measures how strongly your blood is pressing against the walls of your arteries. Arteries are blood vessels that carry blood from your heart throughout your body. You can take your blood pressure at home with a machine.

You may need to check your blood pressure at home:

- To check if you have high blood pressure (hypertension).
- To check your blood pressure over time.
- To make sure your blood pressure medicine is working.

Supplies needed:

- Blood pressure machine, or monitor.
- A chair to sit in. This should be a chair where you can sit upright with your back supported. **Do not** sit on a soft couch or an armchair.
- Table or desk.

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- Small notebook.
- Pencil or pen.

How to prepare

Avoid these things for 30 minutes before checking your blood pressure:

- Having drinks with caffeine in them, such as coffee or tea.
- Drinking alcohol.
- Eating.
- Smoking.
- Exercising.

Do these things five minutes before checking your blood pressure:

- Go to the bathroom and pee (urinate).
- Sit in a chair.
- Be quiet. **Do not** talk.

How to take your blood pressure



Follow the instructions that came with your machine. If you have a digital blood pressure monitor, these may be the instructions:

1. Sit up straight.
2. Place your feet on the floor. **Do not** cross your ankles or legs.
3. Rest your left arm at the level of your heart. You may rest it on a table, desk, or chair.
4. Pull up your shirt sleeve.
5. Wrap the blood pressure cuff around the upper part of your left arm. The cuff should be 1 inch (2.5 cm) above your elbow. It is best to wrap the cuff around bare skin.
6. Fit the cuff snugly around your arm, but not too tightly. You should be able to place only one finger between the cuff and your arm.
7. Place the cord so that it rests in the bend of your elbow.
8. Press the power button.
9. Sit quietly while the cuff fills with air and loses air.
10. Write down the numbers on the screen.
11. Wait 2–3 minutes and then repeat steps 1–10.

What do the numbers mean?

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Two numbers make up your blood pressure. The first number is called systolic pressure. The second is called diastolic pressure. An example of a blood pressure reading is "120 over 80" (or 120/80).

If you are an adult and do not have a medical condition, use this guide to find out if your blood pressure is normal:

Normal

- First number: below 120.
- Second number: below 80.

Elevated

- First number: 120–129.
- Second number: below 80.

Hypertension stage 1

- First number: 130–139.
- Second number: 80–89.

Hypertension stage 2

- First number: 140 or above.
- Second number: 90 or above.

Your blood pressure is above normal even if only the first or only the second number is above normal.

Follow these instructions at home:

Medicines

- Take over-the-counter and prescription medicines only as told by your doctor.
- Tell your doctor if your medicine is causing side effects.

General instructions

- Check your blood pressure as often as your doctor tells you to.
- Check your blood pressure at the same time every day.
- Take your monitor to your next doctor's appointment. Your doctor will:
 - 45 Make sure you are using it correctly.
 - 46 Make sure it is working right.
- Understand what your blood pressure numbers should be.
- Keep all follow-up visits.

General tips

You will need a blood pressure machine or monitor. Your doctor can suggest a monitor. You can buy one at a drugstore or online. When choosing one:

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- Choose one with an arm cuff.
- Choose one that wraps around your upper arm. Only one finger should fit between your arm and the cuff.
- Do not choose one that measures your blood pressure from your wrist or finger.

Where to find more information

American Heart Association: www.heart.org

Contact a doctor if:

- Your blood pressure keeps being high.
- Your blood pressure is suddenly low.

Get help right away if:

- Your first blood pressure number is higher than 180.
- Your second blood pressure number is higher than 120.

These symptoms may be an emergency. Do not wait to see if the symptoms will go away. Get help right away. Call 911.

Summary

- Check your blood pressure at the same time every day.
- Avoid caffeine, alcohol, smoking, and exercise for 30 minutes before checking your blood pressure.
- Make sure you understand what your blood pressure numbers should be.

This information is not intended to replace advice given to you by your health care provider. Make sure you discuss any questions you have with your health care provider.

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Heart Failure Medicines



Heart failure means your heart isn't able to pump enough blood through your body. This can happen if your heart can't fill up with enough blood. It can also happen if your heart is too weak to pump.

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Medicines for heart failure can help your heart pump better and do less work. While there's no cure for heart failure, taking medicines can help you:

- Stay active.
- Have fewer health problems.
- Live longer.

Types of medicines

Your health care provider will look at your symptoms and the type of heart failure to find out which medicine is best for you. In most cases, you may need to take more than one medicine.

ACE inhibitors, ARBs, and ARNIs

- These medicines work to relax the blood vessels. This makes it easier for your heart to work. Angiotensin receptor neprilysin inhibitors (ARNIs) also help your kidneys remove more fluid and salt from your blood when you pee.
- Common angiotensin-converting enzyme (ACE) inhibitors include lisinopril and ramipril. Common angiotensin-receptor blockers (ARBs) include losartan and valsartan. Common ARNIs include sacubitril with valsartan.
 - 46 Only take one type of these medicines. **Do not** combine ACE inhibitors, ARBs, or ARNIs. The risk of side effects grows if more than one is taken at the same time.
 - 46 If you're changing medicines, ask your provider how to do this safely.
- Side effects include:
 - 46 Dry cough. This may happen with ACE inhibitors or ARNIs.
 - 46 Low blood pressure. This may cause you to feel dizzy, light-headed, or have blurred vision.
 - 46 A high potassium level. This may cause muscle weakness or fast or irregular heartbeat.
 - 46 Kidney injury. This may cause you to pee less or swell in your ankles, hands, or feet.
- **Do not** take these medicines if you're pregnant or may become pregnant. They can cause serious birth defects.
- These medicines can cause a serious side effect called angioedema. This is rare. Symptoms include swelling of the face, eyes, tongue, lips, mouth, or throat, trouble breathing, and hoarseness. Stomach pain or swelling and vomiting can also occur.
 - 46 If you have any of these symptoms, stop taking the medicine and contact your provider right away. If you have had angioedema in the past, talk with your provider before starting any of these medicines.
- You may need regular checkups and blood tests to see how the medicine is working.

Beta-blockers

- These medicines treat high blood pressure and heart failure. They may also stop chest pain called angina. They lower blood pressure and heart rate.
- Common beta-blockers for heart failure are bisoprolol, carvedilol, and long-acting, also known as extended release, metoprolol.
- Some beta-blockers make lung diseases that cause wheezing worse. Talk with your provider before taking a beta-blocker if you have a lung disease such as asthma.
- These medicines can hide the symptoms of low blood sugar, also called hypoglycemia. Symptoms include fast heartbeat and shaking that you can't control, called tremors. If you have diabetes, check your blood sugar as told. If you have hypoglycemia, talk with your provider about your medicines.
- This medicine may make you feel dizzy or light-headed. **Do not** drive or use machinery until your provider says that it's safe.
- Because these medicines slow your heart rate, it's important not to do too much exercise. Talk with your provider about what your heart rate should be while you exercise.

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- **Do not** suddenly stop taking this medicine. This can raise your risk of side effects, such as chest pain and heart attack. Your provider will let you know if you no longer need this medicine. They will lower the dose over time to lower the risk of side effects.

Diuretics

- Diuretics treat high blood pressure. They may also be used to reduce swelling related to heart, kidney, or liver disease. They help your kidneys remove more fluid and salt from your blood when you pee. These also reduce fluid in your lungs, ankles, and feet.
- Common diuretics include furosemide, bumetanide, torsemide, metolazone, and hydrochlorothiazide. The amount of water or fluid removed from the body depends on which diuretic is used.
- You may be asked to weigh yourself to see if you have too much or too little fluid. Ask your provider what your weight should be and contact them if it changes. Also, ask how much fluid you should drink each day.
- These medicines can make you pee a lot or make it harder to control peeing. Talk to your provider about the best time of day to take this medicine.
- Side effects include:
 - 46 Headaches.
 - 46 Low blood pressure. This may cause you to feel dizzy, light-headed, or have blurred vision.
 - 46 Low potassium level. This may cause muscle pain or cramps, weakness or tiredness, fast or irregular heartbeat, or constipation.
 - 46 Upset stomach.
- These medicines can cause dehydration. Symptoms include more thirst, dry mouth, feeling faint or light-headed, headache, and dark yellow or brown pee. Contact your provider right away if you have any of these signs.
- These medicines may affect how your kidneys work. This can cause your electrolyte levels (magnesium and potassium) to change. These levels will be checked by your provider.
- These medicines can make you more sensitive to the sun. Keep out of the sun. If you can't avoid being in the sun, wear protective clothing and use sunscreen. **Do not** use sun lamps or tanning beds or booths.

Aldosterone antagonists

- These medicines treat high blood pressure and heart failure. They may also be used to reduce swelling related to heart, kidney, or liver disease. They help your kidneys remove more fluid and salt from your blood through pee without losing too much potassium.
- Common aldosterone antagonists include spironolactone and eplerenone.
- Side effects include:
 - 46 High potassium level. This may cause muscle weakness or fast or irregular heartbeat.
 - 46 Kidney injury. This may cause you to pee less or swell in the ankles, hands, or feet.
 - 46 Low blood pressure. This may cause you to feel dizzy, light-headed, or have blurred vision.
- These medicines can raise the amount of potassium in the blood. Your levels will be checked often by your provider.
- Spironolactone and eplerenone may increase breast size in males and may cause breasts to feel tender in all patients.
- You'll need regular checkups and blood tests to see how this medicine is working.

Digoxin

- Digoxin treats heart failure. It may also be used to treat a type of irregular heartbeat called atrial fibrillation or AFib. It helps your heart beat stronger, making it easier for your heart to pump blood. It also slows down overactive electric signals in the heart. This makes your heart rhythm more steady.
- Your provider will check your digoxin levels with blood tests. Too much digoxin can cause serious problems, such as irregular heart rhythm.

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- Early signs of digoxin toxicity include nausea, vomiting, loss of appetite, headaches, confusion, weakness, or vision changes, such as blurred vision or seeing more yellow or green than usual. Contact your provider right away if you have any of these signs.

Vasodilators

- Hydralazine and nitrates treat heart failure. They relax blood vessels. This makes it easier for your heart to work.
- Hydralazine and nitrates are often given with other medicines to control your heart failure symptoms. You may be given one or both of these medicines, based on your symptoms.
- Side effects include:
 - 46 Flushed face or neck.
 - 46 Headaches.
 - 46 Low blood pressure. This may cause you to feel dizzy, light-headed, or have blurred vision.

If channel blockers

- Ivabradine is an If channel blocker. It lowers your heart rate. This makes it easier for your heart to work.
- If you have symptoms of heart failure and your heart rate is more than 70 beats per minute while taking a beta-blocker, you may be given this medicine.
- Side effects include:
 - 46 Fast or irregular heartbeat, chest pain, or trouble breathing. You may also feel dizzy or light-headed.
 - 46 Slow heartbeat. This may cause you to feel dizzy, light-headed, or confused. You may have trouble breathing, feel weak, or feel very tired.
 - 46 Blood pressure changes. You may have high or low blood pressure. This may cause you to feel dizzy, light-headed, or have blurred vision.
 - 46 Seeing bright or flashing spots, halos around lights, or shifting patterns or colors.
- Grapefruit juice may make the risk of side effects greater. Avoid this as told.
- Talk to your provider if you wish to become pregnant or think you might be pregnant. This medicine can cause serious birth defects.

SGLT2 inhibitors

- Sodium-glucose cotransporter 2 inhibitors, also known as SGLT2 inhibitors, are medicines used to treat type 2 diabetes. They help your kidneys take sugar out of your blood through pee. This lowers your blood sugar. They may also be used to lower the risk of making the disease worse, and of causing death by kidney disease and heart failure. They work by helping your kidneys get rid of salt, also called sodium, from your blood through pee. This lowers the amount of work the kidneys and heart do.
- Common SGLT2 inhibitors include empagliflozin and dapagliflozin.
- Side effects include:
 - 46 Dehydration. This may cause more thirst, dry mouth, feeling faint or light-headed, headache, or dark yellow or brown pee.
 - 46 Diabetic ketoacidosis (DKA). This may cause more thirst, peeing more, dry mouth, tiredness, fruity-smelling breath, trouble breathing, stomach pain, nausea, or vomiting.
 - 46 Genital yeast infection. This may cause redness, swelling, pain, itchiness, odor, or thick or lumpy discharge.
 - 46 Infection or redness, swelling, tenderness, or pain in the genitals, or area from the genitals to the back of the rectum.
 - 46 New pain or tenderness, change in skin color, sores or ulcers, infection of the leg or foot.
 - 46 Urinary tract infection (UTI). This may cause burning when peeing, passing small amounts of pee often, bloody or cloudy pee, or pain in the lower back or sides.
- If you have diabetes, there's a small risk of hypoglycemia. There's also a small risk of DKA. This happens when your blood sugar is very high. DKA will need to be treated in the hospital.

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Heart Failure: How to Manage

Heart failure is a long-term condition where your heart can't pump enough blood through your body. When this happens, parts of your body don't get the blood and oxygen they need.

There's no cure for heart failure, so it's important to take good care of yourself and follow the treatment plan you set with your health care provider. If you're living with heart failure, there are ways to help you manage the disease.

How to manage lifestyle changes

Living with heart failure requires you to make changes in your daily life. Your health care team will teach you about the changes you need to make. These changes can help improve your symptoms and lower your risk of going to the hospital. Work with your provider to create a plan for you.

Activity

- Ask your provider about cardiac rehabilitation programs. These programs include physical activity.
- If no cardiac rehab program is available, ask your provider what exercises are safe for you to do.
- Ask what things are safe for you to do at home. Ask when you can go back to work or school.
- Pace your daily activities and allow time for rest.

Managing stress

It's normal to have many emotions when you're told you have heart failure. You may have fear, sadness, and anger. If you need help coping with any of these emotions, let your provider know. Here are some ways to help yourself manage these emotions:

- Talk to your friends and family about your condition.
 - 46 They can give you support and guidance.
 - 46 Explain your symptoms to them. If comfortable, you can invite them to attend appointments with you.
- Join a support group for people with heart failure.
 - 46 Talking with others who have the same symptoms may give you new ways of coping with your disease and your emotions.
- Accept help from others. **Do not** be ashamed if you need help.
- Use stress management techniques. Try things like meditation, breathing exercises, or listening to relaxing music.

Conditions like depression and anxiety are common in people with heart failure. Pay attention to changes in your mood, emotions, and stress levels. Tell your provider if you have any of the following symptoms:

- Trouble sleeping or a change in your sleeping patterns.
- Feeling sad or depressed.
- Losing interest in activities you normally enjoy.
- Feeling irritable or crying for no reason.
- Often worrying about the future.

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Work

If heart failure affects your ability to work, you may need to talk with your provider about making a plan for changes. This may include:

- Reducing your work hours.
- Finding tasks that require less effort.
- Planning rest periods during work hours.

Travel

- Talk with your provider if you plan to travel. There may be times when your provider suggests that you don't travel or you wait until your condition has improved.
- Bring your medicine and a list of your medicines. If you're traveling by public transportation (airplane, train, bus), keep your medicines with you in a carry-on bag.
- If you have special needs, contact the transportation company prior to traveling. This may include needs related to diet, oxygen, a wheelchair, a seating request, or help with luggage.
- Think about finding a medical facility in the area you'll be traveling to. Find out what your health insurance will cover.
- If you use oxygen, make sure you bring enough oxygen with you.
- If you have a battery-powered oxygen device, bring a fully charged extra battery with you.
- If you have an implanted device, bring a note from your provider and tell the security screening workers that you have the device. You may need to go through special screening.

Sexual activity



- Ask your provider when it's safe for you to resume sexual activity.
- You may need to start slowly and increase intimacy over time.
- Regular exercise can benefit your sex life by building strength and endurance.

Sleep



Arkansas Heart Hospital

Patient Name: MONSON, BERTIA T

MRN: 00213113

FIN: 3650554

DOB/Age/Sex: 5/1/1937 89 years Female

Admit: 5/31/2026

Disch:

Admitting: Hinson MD, Andrew

Discharge Documentation

If your condition affects your sleep, find ways to improve how well you sleep at night. These tips may help:

- Sleep lying on your side, or sleep with your head raised. Try:
 - 46 Raising the head of your bed.
 - 46 Using more than one pillow.
- Ask your provider about screening for sleep apnea.
- Try to go to sleep and wake up at the same times every day.
- Sleep in a dark, cool room.
- **Do not** exercise or eat for a few hours before bedtime.
- Plan rest periods during the day. But don't take long naps.

Where to find support

In addition to talking with your family or friends, consider talking with:

- A mental health professional or therapist.
- A member of your church, faith, or community group.

Other sources of support include:

- Local support groups. Ask your provider about groups near you.
- Online support groups, such as those found through:
 - 46 Heart Failure Society of America: hfsa.org
 - 46 American Heart Association: supportnetwork.heart.org
- Local community agencies or social agencies.
- A palliative care specialist. Palliative care can help manage symptoms, promote comfort, improve quality of life, and maintain dignity.

Where to find more information

To learn more, go to these websites:

- Centers for Disease Control and Prevention at cdc.gov. Then:
 - 46 Click Health Topics.
 - 46 Type "heart failure" in the search box.
- American Heart Association: heart.org
- National Heart, Lung, and Blood Institute: nhlbi.nih.gov

Contact a health care provider if:

- You have trouble breathing when you're active.
- You have swelling in your feet or legs.
- You gain 2–3 lb (0.9–1.4 kg) in 24 hours, or 5 lb (2.3 kg) in a week.
- You have a dry cough.
- You're not able to take part in your usual physical activities.

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- You feel dizzy or light-headed when you stand up.
- You have trouble sleeping.
- You have a decrease in appetite.
- You have symptoms of depression or anxiety.

Get help right away if:

- You have trouble breathing when resting.
- You have trouble staying awake or you feel confused.
- You have chest pain.
- You faint.
- You have fast or irregular heartbeats.

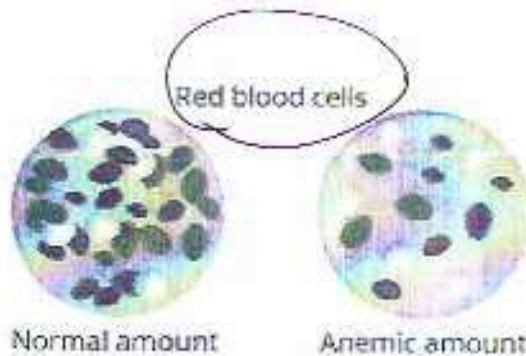
These symptoms may be an emergency. Call 911 right away.

- Do not wait to see if the symptoms will go away.
- Do not drive yourself to the hospital.

This information is not intended to replace advice given to you by your health care provider. Make sure you discuss any questions you have with your health care provider.

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Anemia



Anemia is a condition in which there are not enough red blood cells or hemoglobin in the blood. Hemoglobin is a substance in red blood cells that carries oxygen.

When you do not have enough red blood cells or hemoglobin (are anemic), your body cannot get enough oxygen, and your organs may not work properly. As a result, you may feel very tired or have other problems.

What are the causes?

Common causes of anemia include:

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- Excessive bleeding. Anemia can be caused by excessive bleeding inside or outside the body, including bleeding from the intestines or from heavy menstrual periods in females.
- Poor nutrition.
- Long-lasting (chronic) kidney, thyroid, and liver disease.
- Bone marrow disorders, spleen problems, and blood disorders.
- Cancer and treatments for cancer.
- Human immunodeficiency virus (HIV) and acquired immunodeficiency syndrome (AIDS).
- Infections, medicines, and autoimmune disorders that destroy red blood cells.

What are the signs or symptoms?

Symptoms of this condition include:

- Minor weakness.
- Dizziness.
- Headache, or difficulties concentrating and sleeping.
- Heartbeats that feel irregular or faster than normal (palpitations).
- Shortness of breath, especially with exercise.
- Pale skin, lips, and nails, or cold hands and feet.
- Upset stomach (indigestion) and nausea.

Symptoms may occur suddenly or develop slowly. If your anemia is mild, you may not have symptoms.

How is this diagnosed?

This condition is diagnosed based on blood tests, your medical history, and a physical exam. In some cases, a test may be needed in which cells are removed from the soft tissue inside of a bone and looked at under a microscope (bone marrow biopsy). Your health care provider may also check your stool (feces) for blood and may do more testing to look for the cause of your bleeding.

Other tests may include:

- Imaging tests, such as a CT scan or MRI.
- A procedure to see inside your esophagus and stomach (endoscopy). The esophagus is the part of the body that moves food from your mouth to your stomach.
- A procedure to see inside your colon and rectum (colonoscopy).

How is this treated?

Treatment for this condition depends on the cause. If you continue to lose a lot of blood, you may need to be treated at a hospital. Treatment may include:

- Taking supplements of iron, vitamin B12, or folic acid.
- Taking a hormone medicine (erythropoietin) that can help to stimulate red blood cell growth.
- Receiving donated blood through an IV (blood transfusion). This may be needed if you lose a lot of blood.
- Making changes to your diet.

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- Having surgery to remove your spleen.

Follow these instructions at home:

- Take over-the-counter and prescription medicines only as told by your health care provider.
- Take supplements only as told by your health care provider.
- Follow any diet instructions that you were given by your health care provider.
- Keep all follow-up visits. Your health care provider will want to recheck your blood tests.

Contact a health care provider if:

- You develop new bleeding anywhere in the body.
- You are very weak.

Get help right away if:

- You are short of breath.
- You have pain in your abdomen or chest.
- You are dizzy or feel faint.
- You have trouble concentrating.
- You have bloody stools, black stools, or tarry stools.
- You vomit repeatedly or you vomit up blood.

These symptoms may be an emergency. Get help right away. Call 911.

- Do not wait to see if the symptoms will go away.
- Do not drive yourself to the hospital.

Summary

- Anemia is a condition in which you do not have enough red blood cells or enough of a substance in your red blood cells that carries oxygen.
- Symptoms may occur suddenly or develop slowly.
- If your anemia is mild, you may not have symptoms.
- This condition is diagnosed with blood tests, a medical history, and a physical exam. Other tests may be needed.
- Treatment for this condition depends on the cause of the anemia.

This information is not intended to replace advice given to you by your health care provider. Make sure you discuss any questions you have with your health care provider.

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Smoking Cessation

If applicable, patient received all components of practical counseling including: Recognizing the danger situations, developing coping skills and providing basic information about quitting. Patient received counseling on the benefits of quitting tobacco, how to quit techniques and available resources. Arkansas Tobacco Quitline: One of the most important calls a tobacco user can make is to the Arkansas Tobacco Quitline at 1-800-QUIT-NOW (1-800-784-8669). It is confidential, and it is free. When tobacco users call, they will receive: Free, confidential, non-judgmental expert support from a Quit Coach to help make a plan to quit tobacco.